1. Diagnosis of chronic heart failure

Natriuretic peptide testing is recommended as the initial diagnostic test in patients with suspected chronic heart failure. A plasma NT-proBNP level below 125 pg/mL makes a diagnosis of heart failure unlikely.

Transthoracic echocardiography is recommended in all patients with elevated natriuretic peptides to assess cardiac structure and function and to determine left ventricular ejection fraction.

A 12-lead electrocardiogram is recommended in all patients with suspected heart failure. A completely normal electrocardiogram makes heart failure with reduced ejection fraction unlikely.

2. Classification by ejection fraction

Heart failure with reduced ejection fraction is defined as symptoms and signs of heart failure with a left ventricular ejection fraction of 40 percent or less.

Heart failure with mildly reduced ejection fraction is defined by a left ventricular ejection fraction between 41 and 49 percent together with symptoms and signs of heart failure.

Heart failure with preserved ejection fraction is defined by a left ventricular ejection fraction of 50 percent or greater with objective evidence of cardiac structural or functional abnormalities.

3. Pharmacological management of HFrEF

An angiotensin converting enzyme inhibitor is recommended for all patients with heart failure with reduced ejection fraction to reduce the risk of hospitalisation and death.

A beta blocker is recommended for all patients with stable heart failure with reduced ejection fraction to reduce the risk of hospitalisation and death.

A mineralocorticoid receptor antagonist is recommended for patients who remain symptomatic despite treatment with an ACE inhibitor and a beta blocker.

Sodium glucose co-transporter 2 inhibitors are recommended for patients with heart failure with reduced ejection fraction to reduce the risk of hospitalisation for heart failure and cardiovascular death.

4. Management of acute decompensated heart failure

Intravenous loop diuretics are recommended for all patients with acute heart failure admitted with signs or symptoms of fluid overload to improve symptoms.

Oxygen therapy is recommended in patients with acute heart failure and an oxygen saturation below 90 percent to correct hypoxaemia.

Routine use of intravenous inotropic agents is not recommended in patients with acute heart failure unless the patient has symptomatic hypotension or evidence of hypoperfusion.

5. Device therapy and follow-up

An implantable cardioverter defibrillator is recommended for selected patients with symptomatic heart failure and a left ventricular ejection fraction of 35 percent or less despite optimal medical therapy for at least three months.

Cardiac resynchronisation therapy is recommended for symptomatic patients in sinus rhythm with a QRS duration of 150 milliseconds or more and left bundle branch block morphology.

Patients discharged after an admission for heart failure should be reviewed within two weeks of discharge to assess symptoms, volume status and tolerance of medical therapy.

6. Arrhythmia and rhythm considerations

Frequent premature ventricular complexes in patients with heart failure warrant assessment for a reversible cause and evaluation of left ventricular function.

Anticoagulation is recommended in patients with heart failure and atrial fibrillation to reduce the risk of stroke and systemic embolism.

Routine ambulatory electrocardiographic monitoring is not recommended in asymptomatic patients with heart failure in the absence of a specific clinical indication.